SIMATS ENGINEERING (SAVEETHA SCHOOL OF ENGINEERING)

Department of Computer Science and Engineering

CO1 ASSESSMENT TOOL 2 – Agile Sprint Planning & User Story Workshop

Field	Details
Course Code / Title	CSA10 – Software Engineering
CO Assessed	CO1
Assessment	Assessment Tool 2 – Agile Sprint Planning & User Story Workshop
Weightage / Total Marks	20% / 25 Marks
CO1 Statement	Apply advanced methodologies and agile project management techniques to manage software projects effectively
Student Name: B.Pavani	Reg. No.: 192525248
Date: 16/10/2026	Duration: 60 minutes

Code of Conduct

I B. Pavani (192525248) (Name / Reg No) certify that this submission is my original work and that I have adhered to the guidelines specified for this assessment. I understand that any violation of academic integrity rules will result in disciplinary action.

Signature of the Student: _B.Pavani_

Annexure A – Agile Sprint Planning & User Story Workshop

Instructions to Students:

Each student is assigned an individual software project problem statement. Based on the given problem statement, perform an Agile Sprint Planning and User Story Workshop by applying Agile software engineering practices. Prepare a comprehensive sprint planning document covering the following aspects:

1. **Identify the Project Vision and Stakeholders** – Define the project's vision, objectives, and identify all key stakeholders involved.
2. **Create Epics and User Stories** – Break down the requirements into Epics and formulate well-structured User Stories using the format: As a <user>, I want <feature>, so that <benefit>.
3. **Define Acceptance Criteria** – Write clear and measurable acceptance criteria for each selected user story using the Given–When–Then format or equivalent.
4. **Prioritize the Product Backlog** – Organize and prioritize the product backlog based on business value, customer needs, and project dependencies.
5. **Plan the Sprint Backlog** – Select high-priority user stories for the first sprint and divide them into actionable development tasks.
6. **Estimate Effort Using Story Points** – Assign story points to each selected user story using an appropriate estimation technique (e.g., Fibonacci sequence or Planning Poker) and justify your estimates.
7. **Present the Sprint Goal and Expected Deliverables** – Define a clear sprint goal and describe the expected sprint deliverables, including the features to be completed at the end of the sprint.

Project Title: Remote Patient Monitoring and Predictive Healthcare Platform (Healthcare)

Sprint Planning Document (Sprint 1)

1. Project Vision and Stakeholders

1.1 Project Vision

For healthcare providers managing thousands of patients with chronic conditions, who need to detect health deterioration early, the Remote Patient Monitoring and Predictive Healthcare Platform is a secure, scalable, cloud-based system that continuously collects data from connected medical devices, applies configurable clinical rules and predictive analytics, and alerts clinicians in time to intervene. Unlike fragmented remote care tools, it offers centralized monitoring, longitudinal records, risk prediction and two-way patient engagement in one compliant platform.

Vision statement: Move care from periodic hospital visits to continuous, proactive, data-driven care, reducing readmissions and costs while improving outcomes. **1.2 Project Objectives (SMART)**

#	Objective	Measure of success
O1	Collect patient health data continuously	Device readings ingested and visible within 60 seconds of transmission
O2	Detect abnormal trends early	Abnormal readings flagged in under 30 seconds, with at least 95% rule accuracy against test cases
O3	Notify healthcare professionals	Critical alerts delivered to the care team in under [1] minute, with escalation if unacknowledged
O4	Maintain longitudinal health records	Complete, queryable history per patient with full audit trail
O5	Identify high-risk patients	Risk score generated daily for 100% of enrolled patients
O6	Improve patient engagement	At least 70% of patients log in or sync data weekly
O7	Reduce unnecessary visits and readmissions	Target 20% fewer avoidable visits and 15% fewer 30-day readmissions after 12 months
O8	Ensure security, scalability, compliance	Supports 10,000+ patients, 99.9% availability, encryption, RBAC, audit logs, regulatory compliance (HIPAA principles, India DPDP Act 2023, ABDM guidelines)

Stakeholder	Role	Primary interest
Patients	Primary end users	Easy monitoring, timely care, privacy, fewer hospital trips
Caregivers / family	Secondary users	Visibility into the patient's condition, alerts in emergencies
Doctors / physicians	Primary clinical users	Accurate alerts, risk insights, decision support
Nurses / care coordinators	Primary clinical users	Manage alert queues, follow up with patients
Hospital administrators	Business owners	Lower costs, fewer readmissions, operational reports
Product Owner	Agile role	Backlog value and priority
Scrum Master	Agile role	Process, removing impediments

[1] **.3 Stakeholders**

Development team	Backend, frontend, mobile, QA, DevOps	Clear stories and feasible scope
Data scientists / ML engineers	Technical	Quality data for predictive models
Stakeholder	**Role**	**Primary interest**
Medical device vendors	External	Standard, reliable integration (Bluetooth, API)
IT / Security team	Internal	Secure infrastructure, access control
Compliance / legal officer	Governance	Privacy, consent, regulatory adherence
Regulators (health authorities)	External	Data protection and patient safety
EHR / hospital information system owners	Integration	Interoperable data exchange (HL7 FHIR)
Insurance / payers	Indirect	Outcome and cost evidence

2. Epics and User Stories

Story format: **As a <user>, I want <feature>, so that <benefit>.**

2.1 Epics

Epic	Name	Description
E1	User Management and Access Control	Secure registration, login, roles, consent
E2	Device Integration and Data Ingestion	Collect data from medical devices and apps
E3	Patient Health Monitoring and Records	Real-time vitals, longitudinal health record
E4	Clinical Rules Engine	Configurable thresholds and trend rules
E5	Alerts and Notifications	Abnormality alerts, escalation, acknowledgment
E6	Clinician Dashboard and Care Management	Patient lists, risk views, notes, care plans
E7	Predictive Health Analytics	Risk scoring, deterioration and readmission prediction
E8	Patient Engagement	Patient app, reminders, education, teleconsultation
E9	Reports and Analytics	Health reports, population analytics, exports
E10	Security, Privacy and Compliance	Encryption, audit logs, data protection
E11	Scalability, Reliability and Integration	Performance, availability, EHR/FHIR integration

2.2 User Stories by Epic

Epic	ID	User Story
E1	US-01	As a a system user, I want log in securely with role-based access, so that I see only the data and functions permitted for my role.

E1	US-02	As a nurse or registrar, I want register a patient and record their consent, so that monitoring begins lawfully and the patient is linked to a care team.
E1	US-03	As a administrator, I want manage staff accounts and roles, so that access stays controlled as the team changes.
E2	US-04	As a patient, I want have my connected devices (glucometer, BP monitor, pulse oximeter, smartwatch) send readings automatically, so that I do not have to enter them manually.
E2	US-05	As a patient, I want enter readings manually when I have no device, so that my record stays complete.
E2	US-06	As a administrator, I want register and assign devices to patients, so that each reading is attributed correctly.
E3	US-07	As a doctor, I want view a patient's vitals as a timeline with trends, so that I can see how their condition is changing.
E3	US-08	As a doctor, I want a longitudinal health record (history, medications, readings, alerts, notes), so that I make decisions with the full picture.
E4	US-09	As a doctor, I want configure vital-sign thresholds per patient or condition, so that alerts match each patient's clinical needs.
Epic	**ID**	**User Story**
E4	US-10	As a clinical administrator, I want define trend-based rules (for example, systolic BP rising for 3 consecutive days), so that early deterioration is detected before a threshold is crossed.
E5	US-11	As a nurse, I want be alerted when a reading breaches a rule, so that I can intervene quickly.
E5	US-12	As a care coordinator, I want unacknowledged critical alerts to escalate to a doctor, so that no emergency is missed.
E5	US-13	As a doctor, I want receive alerts by SMS, email and push notification, so that I am reached even when away from the dashboard.
E6	US-14	As a nurse, I want a dashboard of my assigned patients with current status and open alerts, so that I can prioritize my work.
E6	US-15	As a doctor, I want add clinical notes and update care plans, so that interventions are documented.
E7	US-16	As a doctor, I want each patient to have a daily risk score, so that I can focus on high-risk patients.
E7	US-17	As a hospital administrator, I want readmission-risk predictions, so that I can plan follow-ups and reduce readmissions.
E8	US-18	As a patient, I want a mobile or web app showing my readings, trends and goals, so that I understand and manage my health.
E8	US-19	As a patient, I want medication and measurement reminders, so that I stay adherent to my care plan.
E8	US-20	As a patient, I want message my care team or book a teleconsultation, so that I get help without travelling.
E9	US-21	As a doctor, I want generate a patient health report (PDF) for a date range, so that I can share it for consultations.
E9	US-22	As a hospital administrator, I want population-level analytics (cohorts, alert volumes, outcomes), so that I can measure program impact and costs.
E10	US-23	As a compliance officer, I want all access to patient data audit-logged and all data encrypted, so that we meet regulatory requirements.
E10	US-24	As a patient, I want control who can see my data and withdraw consent, so that my privacy is respected.

E11	US-25	As a IT manager, I want the platform to scale to thousands of concurrent patients with high availability, so that service is reliable.
E11	US-26	As a hospital IT user, I want exchange data with the hospital EHR via HL7 FHIR, so that records are consistent across systems.

3. Acceptance Criteria (Given–When–Then)

Acceptance criteria are defined in detail for the 7 stories selected for Sprint 1 (Section 5). The remaining stories will be detailed during backlog refinement before their sprint.

US-01: Secure login with role-based access

#	Given	When	Then
AC1	a registered user with valid credentials	they log in	they are authenticated within 3 seconds and land on a dashboard that matches their role (Doctor, Nurse, Admin, Patient)
AC2	a user enters an incorrect password 5 times	they try a 6th time	the account is locked for 15 minutes and the event is logged
AC3	a Nurse is logged in	they request an Admin-only function (such as user management)	access is denied with HTTP 403 and the attempt is logged
AC4	a user is inactive for 15 minutes	they act again	the session has expired and they must re-authenticate

US-02: Patient registration and consent

#	Given	When	Then
AC1	a Nurse is on the registration form	they submit valid mandatory details (name, date of birth, contact, condition, assigned doctor)	a unique patient ID is created and the patient appears in the care team's list
AC2	the patient's consent has not been recorded	the user tries to activate monitoring	the system blocks activation and shows a consent-required message
AC3	a mandatory field is missing or invalid	the form is submitted	the record is not saved and the specific field errors are shown
AC4	a patient is registered	the record is saved	personal data is stored encrypted and the creation is written to the audit log

US-04: Automatic device data ingestion (Sprint 1 scope)

#	Given	When	Then
AC1	a device is assigned to an active patient	it posts a reading to the ingestion API with a valid device token	the reading is validated, stored and acknowledged (HTTP 201) within 2 seconds
AC2	a reading has an invalid token, malformed payload or out-of-range value (for example, heart rate of 900)	it is received	it is rejected (HTTP 401 or 422), not stored in the patient record, and logged as a data-quality event
AC3	the same reading is sent twice	it is received the second time	it is treated as a duplicate and stored only once

AC4	500 simulated devices send readings concurrently	the load test runs	at least 99% of readings are processed within 5 seconds

US-07: Vitals timeline and longitudinal storage (Sprint 1 scope)

#	Given	When	Then
AC1	a patient has stored readings	a doctor opens the patient's vitals view	readings are shown on a chart in time order with the selectable range 24 hours, 7 days and 30 days
AC2	the doctor selects a parameter (for example, blood glucose)	the chart loads	it renders in under 3 seconds for up to 1,000 data points
AC3	a user without access to the patient	they request the patient's vitals	the request is denied and logged

US-09: Configurable threshold rules (Sprint 1 scope)

#	Given	When	Then
AC1	a doctor opens a patient's rule settings	they set minimum and maximum limits for a vital (for example, SpO2 below 92%) and a severity (Warning or Critical)	the rule is saved and effective immediately
AC2	no patient-specific rule exists	a reading arrives	default condition-based rules (for example, hypertension default set) are applied
AC3	a doctor enters min greater than max	they save	validation fails with a clear error
AC4	a rule is changed	it is saved	the change (who, when, old and new value) is written to the audit log

US-11: Alert on rule breach (Sprint 1 scope)

#	Given	When	Then
AC1	an active rule	a new reading breaches it	an alert with patient, vital, value, threshold and severity is created within 30 seconds
AC2	a Critical alert is created	it is generated	the assigned nurse and doctor see an in-app notification within 60 seconds
AC3	a reading is within normal limits	it is processed	no alert is generated (no false positives in the test set of 50 normal readings)
AC4	an alert is open	a clinician acknowledges it	the alert status changes to Acknowledged with the user and timestamp recorded

US-14: Clinician patient dashboard (Sprint 1 scope)

#	Given	When	Then
AC1	a nurse logs in	the dashboard loads	it lists only their assigned patients with latest vitals, status (Normal, Warning, Critical) and count of open alerts
AC2	patients have different statuses	the dashboard displays	Critical patients appear first and are highlighted in red

AC3	a new alert is raised	the dashboard is open	it updates within 10 seconds without a manual refresh
AC4	500 assigned patients	the dashboard loads	it displays in under 3 seconds and supports search and filter by status

4. Prioritized Product Backlog

4.1 Prioritization Method

Stories are ordered using a combination of:

1. **MoSCoW** (Must, Should, Could, Won't for this release)
2. **Business value** (1 to 10): patient safety impact, clinical value and revenue or cost impact
3. **Dependencies:** technical prerequisites that must be built first (for example, ingestion before rules, rules before alerts, alerts and data before prediction)
4. **Risk reduction:** early validation of device integration and security, which are the biggest risks

4.2 Product Backlog

Rank	ID	Story (short)	Epic	MoSCoW	Value	Pts	Depends on	Sprint
1	US-01	Secure login and RBAC	E1	Must	10	5	None	1
2	US-02	Patient registration and consent	E1	Must	9	3	US-01	1
3	US-04	Automatic device data ingestion	E2	Must	10	8	US-02	1
4	US-07	Vitals timeline and longitudinal storage	E3	Must	9	5	US-04	1
5	US-09	Configurable threshold rules	E4	Must	10	5	US-04	1
6	US-11	Alert on rule breach	E5	Must	10	5	US-09	1
7	US-14	Clinician patient dashboard	E6	Must	9	5	US-11	1
8	US-23	Audit logging and encryption (full)	E10	Must	9	5	US-01	2
9	US-12	Alert escalation	E5	Must	9	5	US-11	2
10	US-13	SMS, email, push notifications	E5	Must	8	5	US-11	2
11	US-06	Device registration and assignment	E2	Must	8	3	US-04	2
Rank	**ID**	**Story (short)**	**Epic**	**MoSCoW**	**Value**	**Pts**	**Depends on**	**Sprint**
12	US-08	Complete longitudinal health record	E3	Must	8	8	US-07	2
13	US-05	Manual reading entry	E2	Should	7	3	US-04	2
14	US-18	Patient app (readings, trends)	E8	Should	8	8	US-07	3
15	US-10	Trend-based rules	E4	Should	8	8	US-09	3
16	US-15	Clinical notes and care plans	E6	Should	7	5	US-14	3
17	US-19	Medication and measurement reminders	E8	Should	7	5	US-18	3
18	US-16	Daily patient risk score	E7	Should	9	13	US-08, US-10	4

19	US-21	Patient health report (PDF)	E9	Should	7	5	US-08	4
20	US-24	Patient privacy and consent control	E10	Should	7	5	US-02	4
21	US-03	Staff and role management	E1	Should	6	3	US-01	4
22	US-26	EHR integration via HL7 FHIR	E11	Should	7	13	US-08	5
23	US-17	Readmission risk prediction	E7	Could	8	13	US-16	5
24	US-22	Population analytics	E9	Could	7	8	US-16	5
25	US-20	Messaging and teleconsultation	E8	Could	6	13	US-18	6
26	US-25	Scalability and HA hardening	E11	Must (ongoing)	8	8	Architecture	2 to 6

Rationale for ordering: Authentication, consent and security come first because healthcare data cannot be handled without them. Device ingestion is next because every other feature depends on data. The rules engine and alerts deliver the core clinical value (early detection and notification), so they precede engagement and prediction features. Predictive analytics (US-16, US-17) are placed later because they need accumulated historical data and a stable rules and record foundation.

5. Sprint Backlog (Sprint 1)

5.1 Sprint Parameters

Item	Detail
Sprint length	2 weeks (10 working days)
Team	5 members: 2 backend, 1 frontend, 1 QA, 1 full-stack/DevOps (Scrum Master and Product Owner in addition)
Assumed velocity	36 to 40 story points (new team, so conservative)
Committed points	36
Ceremonies	Sprint Planning (day 1), Daily Scrum (15 minutes), Sprint Review and Retrospective (day 10)

5.2 Selected Stories and Tasks

US-01: Secure login and RBAC (5 points)

Task	Description	Owner	Est. (hrs)
T1.1	Design user and role schema (Doctor, Nurse, Admin, Patient)	Backend	4
T1.2	Implement authentication API (JWT, password hashing with bcrypt)	Backend	8
T1.3	Implement RBAC middleware and permission checks	Backend	6
T1.4	Account lockout and session timeout	Backend	4
T1.5	Login UI and role-based routing	Frontend	6
T1.6	Write security and unit tests	QA	6

US-02: Patient registration and consent (3 points)

Task	Description	Owner	Est. (hrs)
T2.1	Patient and consent data model (encrypted fields)	Backend	4
T2.2	Registration and consent API with validation	Backend	6
T2.3	Registration form UI with consent capture	Frontend	6
T2.4	Care-team assignment logic	Backend	3
T2.5	Test cases for validation and consent blocking	QA	4

US-04: Automatic device data ingestion (8 points)

Task	Description	Owner	Est. (hrs)
T3.1	Define reading schema (patient, device, vital type, value, unit, timestamp)	Backend	4
T3.2	Build ingestion REST API with device token authentication	Backend	10
T3.3	Validation, range checks and duplicate detection	Backend	6
T3.4	Message queue setup for asynchronous processing	DevOps	8
T3.5	Time-series storage setup for readings	DevOps	6
T3.6	Device simulator to generate test data (glucometer, BP, SpO2, heart rate)	QA/Backend	8
T3.7	Load test with 500 simulated devices	QA	6

US-07: Vitals timeline and longitudinal storage (5 points)

Task	Description	Owner	Est. (hrs)
T4.1	Query API for readings by patient, type and date range	Backend	6
T4.2	Vitals chart component (24h, 7d, 30d ranges)	Frontend	10
T4.3	Access control on patient data	Backend	3
T4.4	Performance and functional tests	QA	5

US-09: Configurable threshold rules (5 points)

Task	Description	Owner	Est. (hrs)
T5.1	Rule data model (vital, min, max, severity, scope)	Backend	4
T5.2	Default rule sets for diabetes, hypertension and respiratory conditions	Backend	4
T5.3	Rule configuration API with validation and audit	Backend	7
T5.4	Rule configuration UI	Frontend	8
T5.5	Rule evaluation unit tests (boundary values)	QA	5

US-11: Alert on rule breach (5 points)

Task	Description	Owner	Est. (hrs)
T6.1	Rule evaluation service consuming the readings queue	Backend	10
T6.2	Alert model and lifecycle (Open, Acknowledged, Resolved)	Backend	5
T6.3	In-app real-time notification (WebSocket)	Backend/Frontend	8
T6.4	Acknowledge alert API and UI	Frontend	4
T6.5	End-to-end alert latency and false-positive tests	QA	6

US-14: Clinician patient dashboard (5 points)

Task	Description	Owner	Est. (hrs)
T7.1	Dashboard API (assigned patients, latest vitals, status, open alerts)	Backend	6
T7.2	Dashboard UI with status colors, sorting, search and filter	Frontend	12
T7.3	Live updates through WebSocket	Frontend	5
T7.4	Usability and performance testing with 500 patients	QA	5

5.3 Capacity Check

Item	Value
Available developer hours (5 members x 10 days x 5 productive hours)	about 250 hours
Total estimated task hours	about 230 hours
Buffer for meetings, reviews, defects	about 8%

The tasks fit the capacity with a small buffer, which supports the 36-point commitment.

6. Effort Estimation Using Story Points

6.1 Technique

The team uses **Planning Poker** with the **Fibonacci sequence** (1, 2, 3, 5, 8, 13, 21). Each member privately selects a card reflecting **complexity, effort, uncertainty and risk** (not hours). Differences are discussed and the team re-votes until it converges.

Reference (baseline) story: US-02, patient registration with a form, validation and a database table, is set at **3 points**. All other stories are sized relative to it. **6.2 Estimates and Justification**

Story	Points	Justification
US-02 Patient registration and consent	3	Baseline. Standard CRUD with validation. Some added work for consent and field encryption, but well understood.
US-01 Secure login and RBAC	5	Security-critical and touches every module. Needs JWT, hashing, lockout, session expiry and role checks. Well-known patterns, so moderate uncertainty, but higher than the baseline because of security testing.

US-04 Device data ingestion	8	Highest complexity and uncertainty in the sprint. Needs a queue, time-series storage, token auth, validation, deduplication and a load requirement. New technology for the team and the foundation for everything else. Not 13, because a simulator avoids real hardware integration in this sprint.
US-07 Vitals timeline	5	Moderate backend querying plus a non-trivial charting component and performance target. Depends on ingestion data.
US-09 Configurable rules	5	Data model, defaults, validation, audit and a UI. Business logic is clear, but boundary conditions need careful testing.
US-11 Alert on rule breach	5	Real-time evaluation, alert lifecycle and WebSocket notification. Moderate technical risk, bounded scope (in-app only; SMS and email deferred to US-13).
US-14 Clinician dashboard	5	Mostly UI and aggregation, with live updates and a performance target. Comparable in size to US-07.
Total	36	Within the assumed velocity of 36 to 40

6.3 Why These Estimates Are Reasonable

- Relative sizing against a known baseline avoids false precision.
- The largest item (8 points) is the riskiest, so the uncertainty is visible and planned for.
- Stories above 8 points (US-16, US-17, US-26, US-20 at 13) are intentionally left in the backlog and will be split during refinement before they enter a sprint.
- Committing to 36 of an assumed 36 to 40 points keeps a buffer, since this is the first sprint and the team has no measured velocity yet. Velocity will be calibrated at the Sprint 1 review.

7. Sprint Goal and Expected Deliverables

7.1 Sprint Goal

> **"Deliver a secure, working end-to-end monitoring slice: a clinician can log in, register a consented patient, receive that patient's device readings, see vitals trends, and be alerted on the dashboard when a reading breaches a configured clinical rule."**

This goal delivers the core value proposition (continuous monitoring, abnormality detection and clinician notification) in its thinnest usable form and validates the highest-risk assumptions (data ingestion, rules and real-time alerting) early.

7.2 Expected Deliverables (Increment)

#	Deliverable	Related story
1	Secure authentication with Doctor, Nurse, Admin and Patient roles	US-01
2	Patient registration with consent capture and care-team assignment	US-02
3	Device ingestion API with validation, deduplication and a device simulator	US-04
4	Time-series storage of readings and a vitals timeline chart (24h, 7d, 30d)	US-07
5	Threshold rule configuration with default condition-based rule sets	US-09
6	Rule evaluation engine and in-app real-time alerts with acknowledgment	US-11
7	Clinician dashboard with patient status, open alerts, search and filter	US-14

8	Test reports (unit, security, load test for 500 devices) and API documentation	All
9	Deployed, demo-ready build in a staging environment	All

7.3 Definition of Done

A story is Done when:

- All acceptance criteria pass in testing.
- Code is peer-reviewed and merged to the main branch.
- Unit test coverage is at least 80% for new code and no critical or high-severity defects remain open.
- Patient data is encrypted, and access is audit-logged where applicable.
- API documentation is updated.
- The feature is deployed to staging and accepted by the Product Owner in the Sprint Review.

7.4 Risks and Mitigation

Risk	Impact	Mitigation
Real device integration is delayed or complex	High	Use a device simulator in Sprint 1; schedule real device (Bluetooth and vendor API) integration for later sprints
Underestimated ingestion and queue setup	Medium	Start US-04 on day 1; pair backend and DevOps; daily progress check
Risk	**Impact**	**Mitigation**
Alert false positives or negatives	High	Boundary-value tests, clinician review of default rules
Compliance gaps (privacy)	High	Encryption, consent and audit logs built from Sprint 1; compliance officer reviews at Sprint Review
Team is new, so velocity is unknown	Medium	Conservative commitment; re-plan using actual velocity

7.5 Sprint Review and Retrospective

- **Sprint Review (day 10):** Live demo using the simulator: register a patient, send normal and abnormal readings, show the alert appearing on the dashboard and being acknowledged. Stakeholders (doctors, nurses, Product Owner, compliance) give feedback.
- **Sprint Retrospective:** Discuss what went well, what to improve and actions, and record the actual velocity to plan Sprint 2 (audit and encryption hardening, escalation, notifications, device assignment, full health record, manual entry).

7.6 Roadmap Preview

Sprint	Focus
1	Core monitoring slice (this document)
2	Escalation, multi-channel notifications, audit and encryption, device assignment, full health record
3	Patient app, trend-based rules, care plans, reminders
4	Risk scoring, reports, privacy controls
5	EHR (FHIR) integration, readmission prediction, population analytics

6	Teleconsultation, performance and scalability hardening, release

Rubrics for Calculation

Criteria	Excellent (5 Marks)	Good (3 Marks)	Satisfactory (2 Marks)	Needs Improvement (1 Mark)
Project Vision & Stakeholder Identification (30%)	Project vision is clear, comprehensive, and aligned with the problem statement. All relevant stakeholders and their roles are correctly identified.	Vision is clear with minor omissions. Most stakeholders are correctly identified.	Vision is partially defined. Some stakeholders are missing or incorrectly identified.	Vision is unclear or incomplete. Stakeholders are poorly identified or missing.
Epics & User Stories (25%)	Epics are well-defined and user stories are complete, follow Agile format, and fully capture functional requirements.	Epics and user stories are mostly complete with minor formatting or requirement issues.	User stories are partially complete with limited Agile structure.	User stories are incomplete, unclear, or do not follow Agile practices.
Acceptance Criteria (20%)	Acceptance criteria are specific, measurable, testable, and aligned with all user stories.	Acceptance criteria are mostly clear and testable with minor gaps.	Acceptance criteria are partially defined and lack clarity or completeness.	Acceptance criteria are missing or not aligned with user stories.
Product Backlog Prioritization & Sprint Planning (15%)	Product backlog is logically prioritized, sprint backlog is realistic, and task breakdown is complete.	Backlog prioritization and sprint planning are mostly appropriate with minor issues.	Prioritization or sprint planning is partially complete with limited justification.	Backlog prioritization and sprint planning are poorly organized or incomplete.
Story Point Estimation, Sprint Goal & Presentation (10%)	Story point estimation is well justified, sprint goal is clear, deliverables are realistic, and presentation is professional and well organized.	Estimation and sprint goal are mostly appropriate with minor inconsistencies. Presentation is clear.	Estimation or sprint goal lacks justification. Presentation is adequate but incomplete.	Estimation is unrealistic or missing. Sprint goal, deliverables, and presentation are unclear or incomplete.

Criteria	Mark
Project Vision & Stakeholder Identification (30%)	/5
Epics & User Stories (25%)	/5
Acceptance Criteria (20%)	/5
Product Backlog Prioritization & Sprint Planning (15%)	/5
Story Point Estimation, Sprint Goal & Presentation (10%)	/5
TOTAL	/25